

Job seeking is a public health issue

The hiring environment as a potential psychiatric exposure

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The hiring industry should regard the conditions of looking for work as a public health concern. Access to income, security, and a viable future can depend on an application process over which the applicant has little control. When scarce opportunities are combined with prolonged uncertainty, repeated rejection, and demands for unpaid effort, the consequences for mental health deserve attention. Employers and job platforms should be expected to examine the burdens their systems create and to test ways of reducing them.

This is what it means to consider job seeking a potential psychiatric exposure: the environment through which people must obtain work may contribute to distress and, for some, mental illness. The relevant exposure includes the availability of suitable jobs, competition for them, and the conditions imposed on applicants. It extends to people searching while employed, including those trying to leave work that is insecure or harmful. The question is how these conditions affect people over time.

An unemployment rate cannot describe this environment on its own. Associated Press reporting on the slow-hiring market in late 2025 described applicants facing canceled recruitment and prolonged searches despite relatively low unemployment. [1] A labor market can look stable from the perspective of those who already have jobs while remaining difficult to enter. For the person seeking work, the uncertainty has practical consequences: how long to keep applying, whether to retrain, which bills can wait, and when an advertised opportunity can reasonably be treated as closed.

I write from within recruitment marketing. That perspective makes the industry's responsibility concrete. Employers and platforms make decisions about the information applicants receive, the effort required of them, and how a search can end. We should account for those decisions in our understanding of mental health. Advice about resilience may help an individual, but it cannot make an unavailable job available or obtain a response from an employer who has stopped communicating.

There is enough evidence to take the exposure seriously

Research has long associated unemployment with poorer mental health, including changes following unemployment and reemployment. Health also affects a person's chances of obtaining and retaining work. [2] The evidence extends beyond the simple distinction between having a job and being unemployed. Charles and DeCicca linked local labor-market conditions to mental health among working-age men in US metropolitan areas, with worse outcomes in groups facing weaker employment prospects. [3] In Spain, Farré and colleagues used the construction collapse during the Great Recession to examine the mental-health consequences of lost employment opportunities in a setting where finding another job was difficult. [4] These studies support treating access to work as part of the health environment.

The experience of searching offers plausible pathways. A daily diary study of unemployed people in China found that searching and distress were related in both directions, with negative search

experiences helping explain subsequent distress. [5] Longitudinal research after job loss linked financial strain and reduced personal control to depression and impaired functioning. [6] Research across three countries also connected perceived unemployment-insurance generosity with mental health through financial strain and time pressure. [7] These findings point toward conditions that institutions can influence: resources, uncertainty, and the amount of control people retain while trying to secure work.

There can also be a damaging feedback loop. In a Norwegian correspondence experiment, applicants who disclosed a history of mental health problems received fewer positive responses from employers. [8] People experiencing distress may therefore encounter additional barriers to employment. A public health approach must consider both the burden of trying to obtain work and the exclusion that can follow poor mental health.

Industry reports give this question a contemporary vocabulary. In a 2024 Resume Genius survey of 1,000 active US job seekers, 72% said searching had negatively affected their mental health at some point in their lives. Forbes subsequently reported the same survey. [9, 10] This was a self-attributed lifetime experience in a selected online sample, so the percentage cannot be read as the current prevalence of a disorder. It does show why applicants' accounts warrant a place in the research agenda.

Greenhouse's survey summary described heightened anxiety and employers ending communication after interviews. Huntr's platform reports describe application activity and experiences during searches. [11, 12] Federal Reserve focus groups likewise documented financial pressure, unstable employment, and barriers to obtaining better jobs. [13] These sources describe burdens that conventional employment statistics seldom capture. They help identify what should be measured, even when their samples cannot establish how common an experience is in the population.

Popular concern about "ghost jobs" belongs in this discussion, with care about what is actually known. An exploratory preprint classified online interview reviews to identify suspected ghost jobs; it did not independently establish employer intent or measure psychiatric outcomes. [14] The useful public health question is whether inaccurate or unresolved information prolongs exposure to uncertainty and wasted effort. That question can be investigated without assuming that every unsuccessful application involved a fictitious vacancy.

Public data establish a burden, but leave the mechanism unresolved

We examined publicly available health and labor-market data to put this argument against evidence. We linked 193,060 out-of-work adults aged 18–64 in the Behavioral Risk Factor Surveillance System (BRFSS) to state hiring, vacancy, and unemployment measures across 2013–2024. Frequent mental distress—at least 14 mentally unhealthy days in the preceding month—was reported by 21.2% of the out-of-work group in 2013 and 28.5% in 2024. [15, 16] Those are substantial burdens. Changing survey coverage and the composition of the out-of-work population mean that the increase cannot simply be attributed to a worsening job market.

We then used regression to ask whether distress changed as a state's market conditions changed, accounting for national year effects and measured demographic differences. The findings were mixed. Lower hiring rates did not show a consistent association with greater distress. More

unemployed people competing for each opening was associated with more distress, but the estimate was uncertain. The brief analysis note below gives the estimates and their confidence intervals. These results do not demonstrate that slow hiring causes illness.

They also identify a limit to what broad indicators can tell us. A statewide hiring rate combines movement between jobs, entry into employment, and other forms of turnover. It does not tell us whether an applicant can find suitable work or whether an employer has left an application unresolved. People entering and leaving the out-of-work group also change its average health. Moreover, unemployment may carry less stigma where it is more common, even as opportunities deteriorate; Clark's panel research illustrates that possibility. [17] A single expected relationship between an aggregate rate and distress is therefore too simple.

The case for attention rests on the wider evidence about opportunity, financial strain, control, and searching. Our analysis adds a reproducible test and an honest boundary: it does not identify the health effects of particular hiring practices. The next step is to measure those exposures directly, including among people who remain employed while searching.

The hiring industry can reduce burdens and test the consequences

We should begin with obligations that follow from asking people to invest time in a recruitment process. Applicants should be able to tell whether a position is currently being filled, what the process will require, and when the employer has ended its consideration. Employers should explain substantial assessments in advance and consider compensation when they demand significant work. Platforms should make it easier to correct or remove stale information and to record when a search opportunity has closed. These are proposed standards for the treatment of applicants; their effects on mental health should be evaluated.

Better information is only part of the response. Clear rejection cannot substitute for suitable jobs, adequate income, or access to care. The evidence on financial strain and perceived income protection makes that distinction consequential. Recruitment reform should accompany efforts to improve economic security. It should not imply that a more considerate application process can resolve a shortage of work.

The industry also has a responsibility to make evaluation possible. With applicants' consent and independent oversight, researchers could follow people from the start of a search, measure symptoms before substantial exposure, and link subsequent changes to local opportunities and application events. Recording delayed responses, repeated assessments, withdrawals, offers, and time spent searching would allow us to ask which experiences precede worsening symptoms. The same study should track employment, earnings, job quality, and who drops out of the process. Better mental-health scores achieved by discouraging disadvantaged applicants would not represent success.

These questions fit an understandable regression framework: do symptoms change after particular conditions or events, after accounting for prior health and relevant changes in circumstances? If financial strain and perceived control are measured repeatedly, a structural equation model could examine proposed pathways. The scientific value would come from measuring the sequence of exposure and response well. Industry partners should contribute access and operational knowledge

while allowing independent analysis and publication, including unfavorable findings. Identifiable mental-health information should never become an input to hiring decisions.

Employers and platforms can also test changes in their own processes, such as predictable status updates or reduced duplication of assessments. The JOBS intervention trial offers a precedent for evaluating employment and mental-health outcomes together, although it tested job-search support rather than platform design. [18] New recruitment practices require their own evidence.

We do not need to assume that every difficult search causes illness to recognize an institutional responsibility. People seeking work depend on systems that govern access to basic resources and shape their ability to plan a future. Treating those systems as potential psychiatric exposures changes the questions the industry must answer: what burdens do we impose, which can we prevent, and does changing them improve people's lives? Those questions belong in public health and in the daily work of hiring.

About the supporting analysis

The primary public-data analysis included 193,060 complete-case out-of-work respondents in 607 state-years, 2013–2024. BRFSS out-of-work categories do not verify active job search, and frequent mental distress is a surveillance measure rather than a diagnosis. Survey-weighted linear regressions included state and year effects, age group, released sex, education, and race/ethnicity. Confidence intervals allowed observations within each state to be related over time. Hiring was the annual mean monthly rate from the Job Openings and Labor Turnover Survey; unemployment came from Local Area Unemployment Statistics. [19, 20]

A one-percentage-point lower hiring rate was associated with -0.80 percentage points in distress (95% confidence interval -2.05 to 0.45), conditional on unemployment. A separate model estimated 1.71 percentage points more distress per doubling of unemployed people per opening (-0.01 to 3.44). Neither interval excluded zero. The opening-rate estimate was also uncertain, and the hiring estimate reversed direction with prior-year exposure. The complete set of models, exclusions, coding decisions, and independent numerical checks is available with the public data and code. [21] A separately labeled extension through 2025 uses the Bureau of Labor Statistics' 11-month unemployment average because October data were not collected during the federal shutdown. [22] These observational estimates cannot establish a causal effect, and state and year effects do not remove all changing confounding. The literature informing this editorial is a focused narrative review; the repository records the review depth and limits of each source.

Author information and disclosures

Jacob E. Thomas is affiliated with Results Generation, a recruitment-marketing firm in Austin, Texas. This industry relationship is relevant to the argument. The views expressed are his own. Correspondence: jacob.thomas@resultsgeneration.com. No external funding supported this work. The analysis used deidentified, publicly released records; no participants were recruited and no proprietary applicant data were used.

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